

WELCOME TO YOUR PREGNANCY JOURNEY

Your guide to a *healthy* pregnancy & thoughtful prenatal care.

Everything you need to know about your visits, your tests,
what to watch for, and the answers to the questions we hear
most often — gathered in one place.

Women's Specialists of Clear Lake

Compassionate care for every stage of womanhood

400 Medical Center Blvd, Suite 300

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281-557-0300 · Fax 281-557-3301

A note from your care team

WELCOME

Congratulations on your pregnancy. Whether this is your first baby or your fifth, we are honored that you've chosen us to care for you. Pregnancy is a remarkable time, and our goal is to make it feel calm, informed, and well-supported from your very first visit through the weeks after delivery.

This packet is yours to keep. It explains what we'll check at each visit, the screening tests we offer and when, the symptoms that should prompt a call to the office, and clear, evidence-based answers to the questions patients most commonly ask. Think of it as a companion — flip to a section when something comes up rather than reading it cover to cover.

The recommendations here reflect current guidance from the American College of Obstetricians and Gynecologists (ACOG) and the CDC. Some details — particularly around timing of tests, aspirin for preeclampsia prevention, and the vaccines now recommended in pregnancy — have been updated to reflect 2026 standards of care.

A few things we want you to know now: call us anytime you are worried. There is no question too small, and we would always rather hear from you than have you wait. Keep this packet handy, bring your questions to every visit, and lean on us when you need to.

With warmth,

Sarah Quintero, RN — and the team at Women's Specialists of Clear Lake

Registered Nurse · 281-557-0300 ext. 24876

How to reach us

Office: 281-557-0300

Sarah, RN: ext. 24876

After hours: call the main line — the answering service will reach the on-call physician.

What to bring to visits

Your insurance card, a list of medications & supplements, prior records or imaging, and any questions you've jotted down between visits.

Your portal

Lab results, after-visit summaries, and non-urgent messages live in your patient portal. Sign up at your first visit.

Your pregnancy at a glance

VISITS & TESTS BY WEEK

A typical visit schedule for a low-risk pregnancy. Your physician may adjust this based on your individual needs. Most prenatal visits include a weight, blood pressure, urine check, listening to the fetal heartbeat, and time to talk through anything new.

8–10 wks FIRST VISIT

BLOODWORK Confirmation, dating ultrasound, full prenatal labs

Complete blood count, blood type and Rh, antibody screen, rubella, varicella, HIV, syphilis, hepatitis B and C, TSH, urine culture, chlamydia and gonorrhea, Pap if due. Expanded carrier screening is offered to every patient.

10–13 wks OPTIONAL SCREENING

SCREENING Non-invasive prenatal testing (NIPT)

A blood draw from you that screens for Down syndrome and other chromosomal conditions and, if you wish, can tell you the baby's sex. Offered to all patients regardless of age, starting at 10 weeks.

12–16 wks IF INDICATED

PREVENTION Low-dose aspirin started

If you have one major or two moderate risk factors for preeclampsia (prior preeclampsia, chronic hypertension, kidney disease, diabetes, autoimmune disease, first pregnancy, age 35+, BMI >30, multiples, or certain ethnic backgrounds), we'll start 81 mg aspirin daily — ideally before 16 weeks — and continue through delivery.

18–22 wks IMAGING

ULTRASOUND Detailed anatomy scan

A thorough look at the baby's anatomy from head to toe at an outside imaging center. Sex can be confirmed if you'd like to know.

20 wks AT HOME

SELF-MONITORING Begin kick counts

Once daily, lie on your side after a meal and time how long it takes to feel 10 movements. Most babies do this in under an hour. Call us if it takes longer or movement feels notably different.

24–28 wks LAB

LAB Gestational diabetes screen

A 1-hour glucose tolerance test. We'll also recheck a CBC for anemia and repeat the antibody screen if you are Rh-negative. RhoGAM at 28 weeks if indicated.

27–36 wks VACCINES

VACCINES Tdap • RSV (when in season) • Flu • COVID-19

Tdap every pregnancy between 27–36 weeks to protect the baby from whooping cough. RSV vaccine (Abrysvo) between 32–36 weeks if you'll deliver in RSV season (September–January). Flu and COVID boosters as recommended.

30–34 wks IMAGING

ULTRASOUND Growth ultrasound

Confirms healthy growth and checks fluid and position heading into the third trimester.

36–37 wks LAB

LAB Group B strep culture

A quick vaginal–rectal swab done between 36 07 and 37 67 weeks. If positive, you'll receive IV antibiotics in labor to protect the baby.

36 wks → delivery WEEKLY VISITS

VISITS Cervix checks, position, labor planning

Weekly visits to monitor blood pressure, fetal position, and signs of labor. We'll review your birth preferences and what to expect.

After delivery RECOVERY

VISITS Early postpartum check & comprehensive visit

An early visit within 3 weeks for physical recovery, mood, breastfeeding, and contraception, followed by a comprehensive visit by 12 weeks.

Call us right away if...

You know your body best. **When in doubt, call.** The main line — 281-557-0300 — connects to the on-call physician day or night.

Vaginal bleeding

Any bright red bleeding, or spotting that soaks a pad, at any point in pregnancy.

Decreased fetal movement

Fewer than 10 movements in an hour after a meal, or a clear change from the baby's usual pattern after 28 weeks.

Regular painful contractions

Before 37 weeks, or coming every 5 minutes for an hour at term.

Sudden swelling

Marked swelling in the face, hands, or one leg — particularly with calf pain or shortness of breath.

Persistent vomiting

Unable to keep anything down for more than 24 hours, or feeling weak and dizzy.

Severe or persistent headache

Especially with visual changes, swelling, or right-upper-abdominal pain — can be a sign of preeclampsia.

Fluid leaking

A gush or steady trickle of fluid from the vagina at any stage.

Fever $\geq 100.4^{\circ}\text{F}$

Especially with chills, urinary symptoms, or upper-abdominal pain.

Severe abdominal pain

Constant, severe, or one-sided pain that doesn't ease with rest.

Mood changes

Persistent sadness, anxiety, intrusive thoughts, or thoughts of harming yourself or the baby — at any point during or after pregnancy.

At every visit, expect

ROUTINE

Standard checks

- Weight and blood pressure
- Urine dipstick for protein and sugar
- Fetal heart rate (after about 10–12 weeks)
- Fundal height starting around 20 weeks
- Review of symptoms, mood, and questions

Visit cadence

- Every 4 weeks until 28 weeks
- Every 2 weeks from 28 to 36 weeks
- Weekly from 36 weeks until delivery
- Earlier or more often if any concerns arise

Quick, evidence-based answers to questions about prenatal care, due dates, and the screenings we offer along the way.

What is prenatal care?

The series of checkups and tests during pregnancy that monitor your health and the baby's growth and catch issues early. Consistent prenatal care is one of the single most important things you can do for a healthy pregnancy.

How is my due date set?

Forty weeks from the first day of your last period, typically confirmed by a first-trimester ultrasound. The ultrasound estimate replaces the period-based one if they differ by more than 5–7 days early in pregnancy.

What is a fetal heartbeat?

The sound of the baby's heart, usually audible by Doppler around 10–12 weeks. Normal rate is 110–160 beats per minute.

What is an ultrasound?

A safe, non-invasive imaging test that uses sound waves to look at the baby. You'll have a first-trimester dating scan in our office, a detailed anatomy scan at 18–22 weeks, and a growth scan around 30–34 weeks. Extra scans may be added if clinically indicated.

What is NIPT?

Non-invasive prenatal testing is a blood draw from you that screens for Down syndrome, trisomy 13 and 18, and certain sex-chromosome conditions. It can also reveal the baby's sex. ACOG now recommends offering it to **all** pregnant patients starting at 10 weeks, regardless of age.

What is carrier screening?

A blood test that shows whether you (and if needed, your partner) carry a recessive gene for conditions like cystic fibrosis, spinal muscular atrophy, or sickle cell. We now offer **expanded carrier screening** — a single panel covering hundreds of conditions — to every patient.

What is amniocentesis?

A diagnostic test performed between 15 and 20 weeks in which a small sample of amniotic fluid is taken to test the baby's chromosomes. Considered when screening is abnormal or risk is elevated.

What is gestational diabetes?

Diabetes that develops during pregnancy when the body can't keep up with insulin demand. Untreated, it raises the risk of a large baby, cesarean, and birth complications. We screen every patient between 24 and 28 weeks, earlier if risk factors are present.

What is Group B strep?

A common bacterium that lives harmlessly in many adults but can cause serious infection in newborns. We swab the vagina and rectum between 36 0/7 and 37 6/7 weeks. If positive, you'll get IV antibiotics during labor — highly effective at protecting the baby.

What is a fetal non-stress test?

A 20–40 minute check in the third trimester where two belts on your abdomen record the baby's heart rate and any contractions. Used when there are concerns about movement, growth, blood pressure, or diabetes.

What is a high-risk pregnancy?

A pregnancy with an increased chance of complications because of your age, medical conditions (hypertension, diabetes, autoimmune disease), prior pregnancy history, or how this pregnancy is developing. High-risk pregnancies get more frequent monitoring and sometimes co-management with a maternal-fetal medicine specialist.

What is advanced maternal age?

Delivery at 35 or older. There is a modestly higher risk of gestational diabetes, hypertension, preterm birth, and chromosomal differences. With good prenatal care, most pregnancies at this age go well.

What is a doula?

A trained non-medical professional who provides physical, emotional, and informational support during labor and the postpartum period. Doulas don't replace your medical team — they complement it. Many patients find their presence reassuring.

What is a childbirth education class?

A short course covering the stages of labor, comfort and pain options, breastfeeding, and newborn basics. Many local hospitals offer them — ask us for recommendations.

Practical, evidence-based guidance on nutrition, exercise, work, and the everyday questions that come up between visits.

What is a prenatal vitamin?

A daily multivitamin formulated for pregnancy. The key ingredients are **folic acid (at least 400 mcg, 600 mcg ideal)**, iron, iodine, choline, and DHA. Start before conception if you can, and continue through breastfeeding.

How much water should I drink?

About 8–12 cups (64–96 oz) of fluid daily, sipped throughout the day. Pale yellow urine is a good sign you're hydrated. Add electrolytes if you're vomiting or sweating heavily.

Can I exercise during my pregnancy?

Yes — and you should. Aim for **150 minutes a week** of moderate activity (brisk walking, swimming, stationary cycling, prenatal yoga). Avoid contact sports, high fall-risk activities, and exercise lying flat on your back after the first trimester. Stop and call us if you have bleeding, fluid leakage, chest pain, severe shortness of breath, or contractions.

Can I continue to work?

Most jobs are safe throughout pregnancy. Talk to us if your work involves heavy lifting, prolonged standing, chemical exposure, or significant fall risk so we can advise on modifications.

Can I travel during pregnancy?

Generally yes, with sensible precautions. Stand and walk every 1–2 hours on long trips to reduce blood clot risk, and stay well hydrated. Most airlines allow flying up to 36 weeks. **Stay close to home after 34 weeks.** Check destination Zika and other travel advisories before booking.

What foods should I avoid?

- Raw or undercooked meat, poultry, eggs, or seafood
- Unpasteurized dairy, juice, or soft cheeses (feta, brie, queso fresco) unless labeled pasteurized
- Cold deli meats and hot dogs unless heated to steaming
- High-mercury fish: shark, swordfish, king mackerel, tilefish, bigeye tuna, marlin, orange roughy
- More than 200 mg caffeine per day (~one 12 oz brewed coffee)
- Any alcohol

What about fish?

Low-mercury fish are **encouraged** — aim for 2–3 servings (8–12 oz) per week. Best choices include salmon, sardines, anchovies, shrimp, cod, tilapia, and canned light tuna.

How much weight should I gain?

Targets are based on your starting BMI — see the weight gain page near the back of this packet. Slow and steady gain in the 2nd and 3rd trimester is the goal, not a number on a single visit.

What medications are safe?

Always confirm with us, but commonly used options include:

- **Pain/fever:** Tylenol (acetaminophen). *Avoid ibuprofen and other NSAIDs after 20 weeks.*
- **Cold/allergies:** Saline nasal spray, Claritin, Zyrtec, Benadryl, plain Tylenol Cold
- **Cough:** Plain Robitussin, Dimetapp
- **Heartburn:** Tums, Pepcid, Prilosec, Nexium
- **Constipation:** Miralax, Colace, Metamucil, Fibercon
- **Diarrhea:** Imodium AD; BRAT diet — call us if >24 hr
- **Hemorrhoids:** Preparation H, Tucks pads, Anusol
- **Nausea:** Vitamin B6, ginger, Unisom, Sea Bands
- **Leg cramps:** Hydration, magnesium-rich foods

Which vaccines are recommended?

- **Tdap** — every pregnancy, 27–36 weeks (protects baby from whooping cough)
- **RSV (Abrysvo)** — 32–36 weeks if delivering September–January
- **Flu** — any trimester during flu season
- **COVID-19** — per current CDC guidance

Live vaccines (MMR, varicella) are not given in pregnancy — get them postpartum if needed.

What should I bring to each visit?

Your insurance card, a current medication list (including supplements), recent records or imaging from outside providers, and any questions you've collected between visits.

Pregnancy comes with a long list of unfamiliar sensations. Most are normal — here's how to recognize and manage the most common ones, and when to call.

Morning sickness

Common in the first trimester, usually improves by week 12–14. Try small frequent meals, ginger or B6, Unisom (doxylamine) at night, and Sea Bands. Call if you can't keep fluids down for 24 hours, lose weight, or feel weak.

Back pain

Driven by posture changes and the growing uterus. Heat or cold, supportive shoes, a maternity belt, prenatal yoga, side-sleeping with a pillow between the knees, and gentle stretching all help. Chiropractic care or physical therapy are reasonable if symptoms persist.

Round ligament pain

Sharp, brief pains in the lower belly or groin from the round ligaments stretching. Move slowly between positions, apply heat, and use a support belt.

Pelvic pressure late in pregnancy

From the baby's head settling into the pelvis. A support belt, side-lying rest, warm baths, and pelvic floor exercises all help.

Symphysis pubis pain

Pain over the pubic bone or hips from softening pelvic ligaments. Avoid wide stances when getting out of bed or a car (keep knees together), use a support belt, and ask about pelvic floor physical therapy.

Constipation

Increase fluids to 8–10 cups daily, add fiber (fruits, vegetables, whole grains, legumes), keep moving, and use Colace 100 mg 1–2 times a day or Miralax as needed.

Shortness of breath

Normal late in pregnancy as the uterus crowds the diaphragm. Sit upright, take slow deep breaths, sleep with extra pillows. Call us for sudden severe breathlessness, chest pain, calf pain, or coughing up blood — those need urgent evaluation.

Dizziness

From blood volume shifts and the uterus pressing on blood vessels. Stand up slowly, stay hydrated, eat regularly, avoid lying flat on your back, wear compression stockings, and sleep on your left side.

Numb hands

Carpal tunnel of pregnancy is common — fluid presses on the median nerve. A wrist splint worn at night, gentle stretching, and avoiding repetitive wrist motion usually help. Most cases resolve postpartum.

Nosebleeds

From increased blood flow and more fragile nasal vessels. Saline spray, a little petroleum jelly inside the nostrils, and gentle blowing help. Pinch closed for 10 minutes if bleeding. Go to the ER if bleeding persists or you feel faint.

PUPPS rash

Itchy, red bumps usually starting on the abdomen in the third trimester. Try cool baths, fragrance-free moisturizers, calamine, hydrocortisone cream, and loose cotton clothing. Severe or generalized itching — especially on palms and soles — needs to be checked for cholestasis.

Bleeding in early pregnancy

Light spotting can be normal (implantation, after intercourse, post-exam). Call us for any bleeding so we can decide whether to evaluate. Heavier bleeding with pain may indicate miscarriage or ectopic pregnancy — go to the ER if you can't reach us.

What are kick counts?

Starting around 20 weeks, lie on your side after a meal and time how long it takes to feel 10 movements. Most babies do this in under an hour. Repeat 2–3 times a week. Call us if it takes longer or your baby's pattern feels different.

A reference for some of the conditions and decisions that may come up later in pregnancy — and what to expect once your baby arrives.

What is preeclampsia?

High blood pressure with signs of organ involvement (typically protein in the urine), most often after 20 weeks. Risk factors include first pregnancy, age 35+, BMI >30, multiples, chronic hypertension, diabetes, and kidney or autoimmune disease. We start **81 mg aspirin daily before 16 weeks** for patients at increased risk to lower it.

What is cholestasis of pregnancy?

A liver condition causing intense itching (often palms and soles), sometimes with darker urine or jaundice. It requires monitoring and earlier delivery to protect the baby. Tell us promptly if you're itching badly.

What is placenta previa?

The placenta covers part or all of the cervix. It often resolves as the uterus grows; if it persists, delivery is by cesarean. Bleeding from previa needs urgent evaluation.

What is preterm labor?

Labor before 37 weeks. Symptoms include regular contractions, low back pain, pelvic pressure, vaginal bleeding, or fluid leaking. Call immediately — there are treatments to slow labor and protect the baby's lungs if delivery is imminent.

What if I'm Rh-negative?

If your blood type is Rh-negative and the baby may be Rh-positive, your immune system can produce antibodies that attack the baby's blood cells in this or a future pregnancy. We prevent that with **RhoGAM**, an injection given around 28 weeks and again after delivery if the baby is Rh-positive.

What if my baby is breech?

Around 37 weeks, if the baby is bottom- or feet-first, we can offer an **external cephalic version** — a procedure to gently turn the baby head-down. If that's not successful or appropriate, delivery is by cesarean.

What are the signs of labor?

Regular, strengthening contractions (every 5 minutes for an hour at term), low back pain, a "bloody show," or your water breaking. Call us — we'll talk through whether to head to the hospital.

How is labor induced?

If induction is recommended (post-dates, hypertension, growth concerns, etc.), methods include cervical ripening (Cervidil or Misoprostol), a Foley balloon, oxytocin (Pitocin), and/or rupturing the membranes. The mix depends on how favorable your cervix is.

What is a cesarean delivery?

A surgical delivery through an incision in the lower abdomen, planned or unplanned. We perform a C-section when it's the safest option for you or the baby — for example, with a previa, certain breech positions, or fetal distress in labor.

What about VBAC?

A trial of labor after a prior cesarean is reasonable for many patients. We'll review your prior records and discuss the risks and benefits, and you'll sign a consent if you choose this path.

What is postpartum hemorrhage?

Heavier-than-expected bleeding after delivery, usually because the uterus doesn't contract well. Risk factors include cesarean, large baby, or prolonged labor. It's managed in the hospital, and your team watches closely.

What is postpartum care?

Care doesn't end at delivery. ACOG now recommends an **early visit within 3 weeks**, with a comprehensive visit by 12 weeks. We check on your recovery, mood, breastfeeding, contraception, and any conditions that arose in pregnancy.

What is postpartum depression?

Sadness, anxiety, irritability, or intrusive thoughts that don't lift after a couple of weeks, or that interfere with daily life. It affects roughly 1 in 7 birthing parents and is very treatable. We screen at every postpartum visit — and you can call any time.

What is a lactation consultant?

A trained professional (often an IBCLC) who helps with positioning, supply, and any breastfeeding problem. Most hospitals offer one before discharge, and we can refer you to an outpatient consultant.

What is a newborn screening test?

A heel-stick blood test in the first day or two of life that screens for dozens of treatable conditions — hypothyroidism, sickle cell, PKU, and more.

About circumcision & cord blood

Circumcision is a personal decision. Benefits include slightly reduced UTIs in infancy, easier hygiene, and reduced HIV/HPV transmission later; risks (bleeding, infection) are uncommon.

Cord blood can be donated to a public bank or stored privately for your family. Public donation is free; private banking carries an ongoing cost with a low likelihood of use. We can refer you if interested.

Healthy weight gain in pregnancy

Gaining the right amount of weight supports the baby's growth and lowers your risk of gestational diabetes, hypertension, and cesarean delivery. The targets below are based on your pre-pregnancy BMI (*weight in kg ÷ height in m²*) and assume a single pregnancy. Twins and triplets follow different targets — ask us.

BMI before pregnancy	Total gain in 1st trimester	Gain per week, 2nd & 3rd trimesters	Goal for whole pregnancy
Underweight BMI < 18.5	2.2 – 6.6 lb	1.0 – 1.3 lb / week	28 - 40 lb
Healthy weight BMI 18.5 – 24.9	2.2 – 6.6 lb	0.8 – 1.0 lb / week	25 - 35 lb
Overweight BMI 25 – 29.9	2.2 – 6.6 lb	0.5 – 0.7 lb / week	15 - 25 lb
Obesity BMI ≥ 30	0.5 – 4.4 lb	0.4 – 0.6 lb / week	11 - 20 lb

Source: Institute of Medicine, *Weight Gain During Pregnancy: Reexamining the Guidelines*; reaffirmed by ACOG.

Eating for two — but just barely

- No extra calories needed in the 1st trimester
- ~340 extra calories/day in the 2nd trimester
- ~450 extra calories/day in the 3rd trimester
- Focus on protein, fruits, vegetables, whole grains, low-fat dairy, healthy fats, and iron-rich foods
- Limit added sugars and ultra-processed foods
- Take your prenatal vitamin daily

Movement that helps

- 150 minutes/week of moderate activity — break it up however works
- Brisk walking, swimming, water aerobics, stationary cycling, prenatal yoga
- The "talk test" — you should be able to hold a conversation
- Stop and call us for bleeding, fluid, contractions, chest pain, or severe shortness of breath

Why this matters

Excess gain raises risk of gestational diabetes, hypertension, large babies, cesarean, and difficulty returning to baseline weight after delivery.

If you're falling behind

Inadequate gain can affect the baby's growth. We'll watch this on your growth charts and may suggest meeting with a dietitian.

If you're gaining quickly

Sudden weight gain with swelling or headache can signal preeclampsia. Call us — don't wait for your next visit.

We're here for *every* step.

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Lab results, after-visit summaries, and non-urgent messages. Ask the front desk to set you up at your first visit.

When in doubt — call us.

We would always rather hear from you than have you wait. **Heavy bleeding, severe headache with vision changes, fluid leaking, fever, decreased fetal movement, persistent vomiting, or thoughts of harm** all warrant a call. The line is open day and night.

*Wishing you a healthy, supported, well-cared-for pregnancy.
— Your team at Women's Specialists of Clear Lake*